

The Surgeon Others Refer To

Positioning and brand strategy for Dr Bola Akinola and Osteon Clinics

The growth constraint is not the postcode. Elective joint replacement is a decision patients make about a surgeon and an institution they trust, not about a convenient address. This note sets out where the white space is, an honest verdict on the location question, and the position we recommend building the brand around.

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1. The headline reframe

You believe the practice is held back by the location of the Amuwo Odofin clinic. The evidence says location is a symptom you have mistaken for the disease.

Elective joint replacement is a decision patients make about a surgeon and an institution they trust, at a price of several million naira, paid out of pocket. It is not a footfall business. The proof is in front of us: Nigerians already board planes to India and Turkey for the exact operation you perform. If distance governed this market, outbound medical tourism could not exist. Patients travel to trust; they do not stay home for convenience.

So the real constraints are three, and none is the address:

- **A thin, un-engineered referral pipeline**, which is the actual engine of elective specialist volume.
- **A personal and institutional brand that badly under-sells a world-class CV.**
- **Leakage to outbound medical tourism** that you are positioned to recapture but are not marketed to capture.

The move is not to relocate. It is to decouple the brand from the postcode: build Osteon into a named centre of excellence, and lead with a position no competitor in Lagos has claimed.

Our recommendation: position you as the complex-case and revision authority, the surgeon other surgeons refer to when a case is hard or has gone wrong. This is your genuine subspecialty (revision arthroplasty, bone and joint infection, complex trauma and limb reconstruction), it is unoccupied by every competitor we mapped, it is the hardest position to fake and therefore the highest in trust, and it is the strongest referral magnet there is: surgeons who send you their failures also send you their primary cases. It reframes medical tourism in your favour: not “do not go abroad,” but “when abroad goes wrong, this is where you come home.”

That spearhead is carried by an ethics-forward voice, the surgeon who will tell you honestly when not to operate, powered by your public-health training. In a market whose defining problem is a trust deficit, honesty is a differentiator, not a soft skill.



2. The market: real, large, and getting more winnable

- **The leakage is big.** Nigeria loses between roughly US\$685m (narrowly tracked) and a widely cited about US\$2bn a year (medical-association advocacy estimate) to outbound medical travel. Orthopaedics is consistently named among the top leakage specialties.
- **The reported collapse in outbound spend is a mirage.** Tracked medical-tourism FX fell about 96 percent year-on-year, but health-sector leaders attribute this to payments moving to untracked channels after FX convergence, not to patients staying home. The outflow continues; it has gone invisible to the data.
- **The economics of flying out have turned, and this is the real tailwind.** India markets knee and hip packages at about US\$4,000 to US\$8,000, which post-devaluation is roughly N6m to N13m in surgery alone, before flights, visas and accommodation for the patient and a companion, pushing the true cost of a trip abroad well past N10m. Local arthroplasty is not cheap either: the floor in Nigeria is now around N5.5m post-devaluation, with premium and complex cases higher. But even so the strong local surgeon is now both better value and more convenient. The high local price cuts the other way too: at N5.5m and up, cost prices most patients out, which is why access and affordability matter as much as reputation.
- **The barrier is trust, not price or competence.** This is the single most important finding: once patients have confidence in local quality, safety and demonstrated outcomes, the reason to travel disappears. A trust barrier is a branding barrier, which is exactly the problem we have been engaged to solve.

Two channels are opening:

- **Diaspora-doctor batch surgery.** Nigerian specialists abroad partner with local hospitals to complete blocks of complex procedures, explicitly including advanced orthopaedics. A locally based, UK-credentialled surgeon can anchor this rather than cede it to fly-in visitors.
- **Diaspora-funded care.** Adult children abroad paying for a parent's surgery at home. This buyer is often more reachable online than the patient, and responds strongly to a recognisable UK credential and clear outcomes.

3. The competitive map: two flags planted, a wide-open centre

Player	Position they own	Gap they leave open
Dr Julius Oni / O.N.I. Clinic	"World-Class Orthopedics, Here at Home." The reverse-medical-tourism flag, on Johns Hopkins pedigree and 4,000+ procedures.	A volume-and-speed, borrowed-prestige play. Thin on complex, revision and salvage.
Dr Seyi Idowu / Lagos Ortho Clinic	Public-institution anchor (National Orthopaedic Hospital, Igbobi) plus musculoskeletal oncology and tumour surgery.	Not a premium private brand; oncology focus, not revision or infection.
Vedic / Duchess / Starcare / Indian inbound	Facility-led or price-led packages.	Commoditised; no named-surgeon trust equity.

Two conclusions follow, and they set the whole strategy:



- **“World-class at home” is already Oni's territory.** You cannot lead with it without becoming a me-too against a stronger-branded Johns Hopkins name on the Island. The market is real; the slogan is taken.
- **Nobody has claimed the complex-case and revision authority.** No premium Lagos brand foregrounds revision arthroplasty, bone and joint infection, and complex trauma and limb reconstruction, the cases that go wrong elsewhere. That is precisely your subspecialty and your BOA arthroplasty-and-trauma fellowship.



4. The location verdict

For elective, high-value specialist surgery, the evidence points away from geography as the constraint. The referral-science literature finds specialist referral turns on three things: the specialist's reputation; patient-interaction factors, of which convenience is only one sub-factor; and the referrer-to-specialist relationship (trust, communication, institutional affiliation, reciprocity). Referrers rely on subjective trust and relationships, not outcome data. That means the pipeline is built by relationship work, not real estate. The one study that looks pro-location is US routine-surgery data, and it is contradicted by Nigerians travelling six hours to India for this same operation.

One honest caveat. The arthroplasty literature does say institutional reputation matters alongside surgeon reputation. Patients do not care about the Amuwo postcode, but they do care whether the place reads as a credible centre of excellence. So the answer is not “personal brand only,” and it is definitely not “move.” It is to turn Osteon into a recognisable institutional brand of arthroplasty excellence, so the address stops mattering. Location is a brand-equity problem wearing a real-estate costume.

What to do instead of relocating:

- **Formalise the hub-and-spoke visiting model** already in place (Cedarcrest Lagos, Diamed) so the brand and the surgeon travel to where the patients and referrers are, with Osteon as the named home base.
- **Engineer a referrer engine:** GPs, physiotherapists, rheumatologists, and above all other orthopaedic and trauma surgeons who refer their complications. The complex-case position is the single best tool for this.
- **Add a concierge patient pathway** (pre-op guidance, transport, diaspora liaison) so the journey to Amuwo feels premium and considered, not peripheral.



5. Recommended positioning

We recommend a layered position, not an either/or: a spearhead niche, carried by a trust-first voice, with a deliberate touch of world-class care at home as the reassurance that closes.

5.1 The spearhead: the complex, revision and salvage authority

Position you as the surgeon other surgeons refer to. The authority for the cases others cannot or will not take: failed and infected joint replacements, revision arthroplasty, non-union and complex trauma, limb reconstruction, and pointedly, botched-abroad cases that come home to be fixed.

- **It is genuinely yours.** This is naming what you already are, not inventing a claim.
- **It is unoccupied.** No competitor we mapped foregrounds it.
- **It is the highest-trust position available.** Complex and salvage work is the hardest to fake, so the claim carries weight a volume number cannot.
- **It is the strongest referral magnet.** Surgeons who trust you with their failures send you their primaries too. The complex position is the lighthouse; the primary practice is the harbour it fills.
- **It reframes medical tourism in your favour.** Oni's volume-and-speed brand cannot credibly say "when your surgery abroad goes wrong, come home to me." You can.

5.2 The voice: right care, honestly

Wrap the spearhead in an ethics-forward, outcomes-and-integrity tone, powered by your MPH: the surgeon who will tell you honestly when you do not need the operation. Against price-driven package sellers and volume players, "I will tell you the truth about whether you need this surgery" is a powerful and defensible differentiator, and it makes every recommendation to operate more believable.

5.3 A touch of world-class care at home

Keep a deliberate touch of "world-class care, at home" running through the brand, but do not lead the whole practice on it, because Oni already owns that flag as a general promise. The differentiation is in what we attach it to: world-class care at home for exactly the cases you would otherwise fly abroad for, the revision, the infection, the complex reconstruction. That turns a crowded slogan into a sharp one. In practice: the complex-case position is the headline, the honest-counsel voice is the tone, and world-class-care-at-home is the reassurance that closes the diaspora child and the anxious patient.

5.4 Brand architecture: two separate brands, one clear hierarchy

You own Osteon Clinics, but the two are deliberately kept as separate entities and separate brands. That is the right structure, and the strategy leans into it. The hierarchy is explicit:

- **Dr Bola Akinola is the primary, hero brand.** The named surgeon is where the marketing investment, the personal story, the credentials and the complex-case authority live. He is the draw, the trust anchor and the referral magnet, and the person a referring doctor or a diaspora child is actually choosing. bolarinwaakinola.com is the flagship.
- **Osteon Clinics is the secondary, supporting brand.** It is the institution that delivers the care and supplies the centre-of-excellence credibility patients also weigh: standards, theatre, team, outcomes, pathway. Endorsed by your reputation, but a distinct brand you own.

Why keep them separate rather than merge them:



- **The personal brand carries what cannot scale; the institution carries what can.** A complex-case, high-trust reputation is inherently personal, which is why it commands premium and generates referrals, but it caps at your own hands. Osteon, kept separate, can later carry what your name should not stretch to: primary-elective volume, other surgeons, allied services, insurance-friendly lines, more sites, growing beyond and outlasting any single surgeon without diluting your specialist positioning.
- **It protects the premium.** If the hero brand and the institution are one, every routine and price-led line drags on the specialist reputation. Separated, your name stays scarce and elevated, while Osteon does the broader, more accessible work under its own name.

How they hand off. Your name draws the complex or referral patient in and earns the trust; Osteon delivers the care and reassures the patient that the place, not just the person, is world-class, which is what keeps them in Nigeria. Marketing leads with the man; delivery and institutional proof are branded Osteon. In copy this reads as “Dr Bola Akinola at Osteon Clinics,” never “Osteon” alone for the specialist work.

The flagship site (bolarinwaakinola.com) currently under-sells all of this. It leads on a generic “care you can trust” line and buries the revision, infection and complex-trauma subspecialty that is the moat. No visible outcomes, proof points, testimonials or referrer pathway. The rebuild should lead with the complex-case authority under your name, present Osteon as the endorsed place of delivery, surface proof, and add referrer and diaspora pathways.



6. What we need to confirm before we build

Four things the research left open, best answered from your own numbers and a few market calls. Your answers turn this strategy into an execution plan.

#	Question we need answered
1	Payer mix and price band for your cases: self-pay versus diaspora-funded versus any HMO, and how price-sensitive the self-pay segment is.
2	Your actual case mix today: what share is already complex or revision versus primary elective. This tells us whether the position is a reposition or simply naming what you already do.
3	Which referral sources already convert, and whether the Cedarcrest and Diamed visiting work already out-produces the Amuwo base. A direct test of the location verdict.
4	The remaining competitors' positioning (Vedic, Duchess, Cedarcrest as a brand) to confirm the centre is as open as it appears.

Suggested next step: a short discovery questionnaire covering items 1 to 3, plus a 45-minute working session. On the answers, we lock the positioning, then move to the brand and website build.

A note on the evidence base. The market, competitor and medical-tourism figures come from a structured research exercise with each central claim independently fact-checked. Two things to hold lightly: the headline medical-tourism spend figures are advocacy estimates, not audited data, and the strongest referral-behaviour studies are international and applied here by inference. Directional, not precise.

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